
CONSULT FOR AFRICA

Arabella Women's Health

Diagnostic Audit: Information and Data Request

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For Dr Chito Nwana and the Arabella Women's Health leadership

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Lagos and Abuja, Nigeria



A note before the list

Dr Chito, thank you for confirming the engagement. We start with you on Friday and I want the first week to be worth something to you rather than a week of us finding our way around, so this list is what we need in order to arrive already working.

You said the gap has never been strategy, it has been execution, and I agreed with you in writing. That agreement is what shapes this document. We are not coming to write you a view of the Abuja women's health market. We are coming to establish what Arabella actually earns today, where the money and the patients leak out between the first enquiry and the bank account, and whether the operation can carry the premium promise the brand is about to make. Everything below serves one of those three questions.

I want to say three things plainly up front.

Nothing in this list is a test. Where a record does not exist, tell us it does not exist. The absence is itself a finding and it is almost always a more useful one than a tidy file would have been, because it tells us where the business is running on memory rather than on a system. We would rather see the real spreadsheet, the WhatsApp group where bookings get confirmed and the notebook at the front desk than a version assembled for our benefit. Assembling it costs your team a week and tells us nothing about how Arabella runs on an ordinary Tuesday.

The second is about the transition, and it is the thing I most want to look at carefully. Arabella is a new legal entity with its own RC number and Tabitha is the business it succeeds. That succession is being asked to carry a great deal of weight all at once: the HMO panels are to treat Arabella as the continuation of Tabitha for tariff purposes, the patients are to meet Arabella as something new and more premium, and the reactivation of the existing patient base depends on the new company being able to contact the old company's patients lawfully and without it feeling like a cold call. Those three claims need to rest on one consistent set of documents. Section A exists to find out whether they do. If they do not, we would all rather know that in week one than in month four when an HMO medical director asks.

The third is confidentiality. Your numbers, your case records, your tariffs and your staff's answers do not leave Consult for Africa. They are not shared with any other client of ours, they are not shared with any hospital or HMO except in the specific documents you approve and sign, and nothing is disclosed to any third party unless you tell us in writing.

Warm regards,

Dr Debo Odulana

How to send it

Please nominate one point of contact who can chase the rest of the team. Tolu is the obvious choice if that works for her, and it helps us if requests go through one person rather than through you.

Everything for this audit lives on one private page: consultforafrica.com/ArabellaProject. It carries this document, an uploader that takes files straight from a phone or a laptop and tags them to the section they answer, and the four short surveys. Send things as you find them rather than waiting until you have everything, and come back to it as often as you like. The page is not listed anywhere and is not indexed. If you would rather use a shared folder in Google Drive or OneDrive, that is equally fine: make one with a sub-folder per lettered section below and give us access to it, so that nothing is lost in a thread.

Exports from a system beat typed summaries every time. A raw CSV or Excel export from the billing software, the accounting package or the appointment book is worth more to us than a neat table someone had to retype, and it carries no transcription error. Photographs of paper registers are fine and often better than nothing. Read access to a system, even for one week, is better than any export at all.



Where something is commercially or personally sensitive, mark it and send it anyway, because it is handled under the confidentiality above. Where a question is easier answered in conversation than in writing, write "discuss" against it and we will cover it on site.

Before Friday: the eight that matter most

These eight unlock everything else and they are what we will be working from while we are with you. If the week runs away, send these and nothing else.

#	What we need	Why it is first
1	Arabella's certificate of incorporation and RC number, and the CAC status report	Nothing in the payer workstream can begin without it, and every introduction letter quotes it
2	The list of Tabitha's HMO panels, with tariff schedules and the contact name at each	It is the single most valuable asset in the transition and the hardest to rebuild if it is lost
3	The revenue record, monthly, last 12 months, split by service line and by payer	This becomes the agreed baseline, and the baseline is the number every later judgement is measured against
4	Bank statements for every business account, last 6 months	The only unarguable record of what actually came in, as opposed to what was invoiced
5	The current price list and package architecture: maternity, gynaecology surgery, fertility	We cannot see yield or leakage without knowing the list
6	The patient database export, in whatever form it exists	The reactivation case rests on it, and its quality is unknown until we open it
7	Full staff list with role, employment type, days worked per week and reporting line	Establishment against service is where specialist centres quietly fail
8	Whatever facility registration and licences are held, including for the fertility service	Category registered against service delivered is a live exposure, and it is worse in a new entity

A. The two entities, and what the succession actually rests on

This section is short and it matters more than its length suggests. Please treat it as the priority after the eight above.

- Arabella's certificate of incorporation, memorandum and articles, CAC status report and RC number
- Tabitha Medical Centre's equivalent documents, and confirmation of whether that entity remains in existence, dormant or dissolved
- The relationship between the two in writing: any business transfer, asset purchase, licence or novation agreement, however informal, and if none exists, say so
- Who owns the premises, and under which entity the lease or title sits
- Which assets moved and which did not: equipment, stock, the website and domains, the social media accounts, the phone numbers, the bank accounts
- Which contracts moved: staff, suppliers, HMOs, landlord, software, the pharmacy wholesaler
- The staff position: were employees terminated and rehired, transferred, or simply continued, and what were they told in writing



- **Custody of the patient records.** Under which entity were the existing records created, under which are they now held, on what basis did they move, and what were patients told
- Any tax, PAYE, pension or regulatory liability left behind in the old entity, and any outstanding creditor
- What has already been said to HMOs, patients, staff and suppliers about the change, and in what form
- Whether the two names are being used interchangeably anywhere today: signage, invoices, receipts, prescriptions, referral letters, the ultrasound report header

B. Ownership, governance and how decisions get made

- Current shareholding of Arabella, and whether any shares are held by a non-Nigerian person or entity
- Directors, and any shareholders' agreement
- Any other company you own or control that touches this work, including any property, equipment or supply entity
- How the business is governed today: who meets, how often, what is minuted, who signs off spending above what threshold, and who can commit the company
- Any management or board meeting notes from the last 12 months
- Bank mandate: who are the signatories, and what is the approval route for a payment
- Any loan, overdraft, equipment finance, related party balance or personal money put into the business, with terms
- Your own position: what you are paid, how it is drawn, and whether clinical income you earn elsewhere is invoiced through Arabella or personally
- Any other clinician with an economic interest, a revenue share or a sessional arrangement

C. Licensing, regulatory standing and insurance

- Private health facility registration for the premises with the FCT health authority: the certificate, the category the premises is registered under, the expiry date, and any conditions attached
- Any inspection report or correspondence from the regulator, and any undertaking given
- The named medical director of record for the premises, and confirmation that the registration is not also being relied on by any other facility
- MDCN full registration and current annual practising licence for every doctor who works here, including visiting consultants, anaesthetists and locums
- Nursing and Midwifery Council licences for every nurse and midwife, including theatre, recovery and postnatal staff
- **The fertility service specifically:** who the responsible clinician is, who the embryologist is and what their qualification and registration are, what written guidance the unit operates to, and any accreditation or professional body affiliation held
- **The embryology laboratory and cryostorage:** the consent forms used for gametes and embryos, what patients are told about storage duration and disposal, the alarm and monitoring arrangement on the tanks, the liquid nitrogen supply and its backup, and what happens on a power failure
- Theatre and anaesthesia: who provides anaesthetic cover, on what notice, and what the arrangement is out of hours and at weekends
- **The obstetric escalation route, in writing:** the named unit you transfer to for intensive care and for neonatal intensive care, the agreement or understanding behind it, the transport arrangement, and the last three times it was used



- Blood: the transfusion arrangement, the supplying bank or service, cross matching, and how long blood takes to reach theatre in an obstetric haemorrhage
- Sterile processing: the autoclave, its validation and load records, how an instrument set is traced to a patient, and who is responsible
- Pharmacists Council premises licence and superintendent pharmacist, since medicines are dispensed on site
- Controlled drugs: the register, the storage, who holds the key, and the last reconciliation
- Medical Laboratory Science Council registration for any laboratory work done on site
- Radiation: the Nigerian Nuclear Regulatory Authority licence for any X-ray unit, plus the radiation protection officer and dosimetry records, if any such equipment is held
- Professional indemnity for you, for every other clinician, and for the facility, with sums insured and expiry dates
- Public liability, fire, burglary and equipment insurance
- Clinical and sharps waste: the contract, the carrier, and the consignment notes
- Fire safety certificate, environmental permit, and any local or signage permits
- Data protection: whether the company is registered with the Nigeria Data Protection Commission under the Act, who the data protection officer is, and what patients are told about their records
- Any current or threatened claim, regulatory complaint or litigation, and any patient complaint that escalated beyond the clinic

D. Money: what comes in and what it costs to stand still

- Monthly management accounts or profit and loss for the last 24 months if they exist, 12 at minimum, for both entities where the period spans the change
- If formal accounts are not prepared, the income and expenditure record in whatever form it is kept, including a personal notebook
- Latest balance sheet, trial balance and chart of accounts
- Bank statements for every business account for the last 12 months, and confirmation of any personal or staff account through which business receipts have passed
- The accounting system in use, and read access or a full export
- Any accountant prepared or audited statements, and the current position on PAYE, VAT, pension remittance and company income tax for both entities
- Fixed asset register with purchase dates and values, and which entity owns each item
- **The monthly fixed cost run rate, itemised:** rent, salaries by department, diesel and power, water, internet, security, cleaning, software, insurance, waste, professional fees
- Any budget or plan for this year, and actuals against it
- The cash position today, the payables list with ages, and the worst month of the last two years with the explanation
- Cash handling: who takes money at the front desk, what the receipting process is, when it is banked, and who reconciles it against the day's activity

E. The service ledger: volume, mix and what each episode earned

This is the most important dataset in the audit. One row per patient episode, last 24 months, across every service line. Whatever fields exist is fine. Please do not create fields that do not.



- Date, service line, procedure or package, and whether elective or emergency
- Patient age, and a local identifier so that we can trace a sample on site. Names are not needed for the analysis
- Clinician, assistant, anaesthetist and midwife where relevant
- Payer: self pay, HMO and which one, corporate, family abroad, or staff and courtesy
- **Amount quoted, amount invoiced, amount collected, and any discount or write off with the reason**
- The direct cost captured for that episode: consumables, drugs, implants, laboratory, outsourced imaging
- Length of stay, and whether discharge was as planned
- Outcome, and any return to theatre, readmission, transfer out or complication
- Separately, for maternity: bookings taken, deliveries, caesarean rate, and how many booked patients delivered elsewhere and why
- Separately, for fertility: cycles started, cycles cancelled before transfer, transfers, and clinical pregnancies, by age band
- Separately, for gynaecology surgery: procedures listed, procedures cancelled on the day, and the reason for each cancellation

F. Pricing and the package architecture

- The current full price list, every service, and the date it was last changed
- **What a maternity package includes and, precisely, what it excludes.** What happens commercially on a caesarean, a prolonged stay, a neonatal admission, a transfer out, or a twin pregnancy
- How a package is paid: deposit, instalments, the point at which the balance is due, and what happens when a patient stops paying part way through a pregnancy
- How a surgical quote is built, who builds it, and who may vary it
- Fertility pricing: what a cycle includes, how drugs are charged, how a freeze or a subsequent transfer is priced, and what a cancelled cycle costs the patient
- Any published or advertised price, anywhere, including on the website, in a brochure, on social media or quoted in a message
- The last five quotes issued that did not convert, with whatever is known about why
- Any comparison you have already done against other Abuja facilities, formal or informal

G. Payers: HMO, corporate and self pay

- The list of every HMO panel held by Tabitha, with the tariff schedule, the effective date, the contract, and the name and number of the contact at each
- Which panels are active, which lapsed, and which were never formalised beyond an understanding
- **HMO receivables:** what is outstanding, by payer, by age
- Claim rejections and short payments over the last 12 months, with the stated reasons, and who chases them
- The authorisation process today: who obtains a code, how long it takes, and what the front desk does when it does not come
- Any corporate or embassy account, retainer or arrangement, current or lapsed, with the terms
- The self pay share of revenue, and how it has moved over the last 12 months



H. The patient database and the right to contact it

- The full export of the existing patient database, with every field it holds
- How it was built, what system it sits in, when it was last cleaned, and who has access to it today
- How many records carry a working phone number, how many carry an email, and how many carry a usable clinical history
- **On what basis each patient's contact details were collected, and what they were told they would be used for.** If a consent or privacy notice was ever signed or displayed, we need to see it
- Whether any patient has asked not to be contacted, and how that is recorded
- Any outreach, campaign or broadcast already sent to this list, when, through what channel, and what came back
- Whether the list has ever been shared with, or is held by, any third party including a former marketing agency or an individual staff member's personal device

I. Clinical governance, safety and the patient journey

- Whatever clinical protocols exist in writing, and an honest indication of which are followed
- The protocols for the emergencies that actually happen here: postpartum haemorrhage, pre-eclampsia and eclampsia, shoulder dystocia, neonatal resuscitation, and sepsis
- The resuscitation trolley contents and the last check, and who is currently trained in newborn and adult life support with expiry dates
- The maternal and neonatal record: what is recorded, where, and by whom
- Consent forms in use for delivery, surgery and fertility treatment
- Incident reporting: whether it exists, what has been reported in 12 months, and what changed as a result
- Any mortality, stillbirth, near miss or unexpected transfer in the last 24 months, and what review followed
- Infection prevention: the last audit if any, hand hygiene, theatre discipline, and cleaning schedules with sign off
- The patient journey as it actually runs, from the first enquiry to the follow up visit: who answers the phone, how a booking is made and confirmed, what happens when a patient arrives, how long they wait, who takes payment, and who follows up afterwards
- Any patient feedback, review or complaint held, in any form, including social media comments

J. Pharmacy, stock and the formulary

- The current stock list with quantities, unit costs and expiry dates
- The formulary, if one exists, and how an item gets onto it
- Purchase records for 12 months with supplier, volume and price
- Pharmacy revenue and margin, monthly, and how the selling price is set
- Stockouts in the last three months, what they were, and what the patient was told
- Expiry write offs in 12 months
- Who orders, who receives, who counts, and when stock was last physically counted against the record
- Cold chain: what requires it, what monitors it, and what happens on a power failure
- Any supplier credit line, and anything owed



K. People: establishment, roles and accountability

- Full staff list: name, role, qualification, employment type, days and hours worked, reporting line, start date and gross pay
- Organogram as it is meant to be, and a note where it differs from how the place actually runs
- Contracts of employment, job descriptions, and the staff handbook or policies if any
- The rota for the last three months, and how a night, weekend or public holiday is covered
- Who covers the front desk, and who covers it when that person is away
- Appraisal, supervision or performance conversation records, if any
- Turnover over 24 months: who left, from which role, and why
- Training records, and any mandatory training tracked
- **How tasks are assigned and checked today.** Who decides what needs doing, how the person doing it knows, and how anyone knows it was done
- Payroll cost by department, monthly, and any bonus, commission or incentive arrangement in place

L. Technology, records and information security

This section supports the technology and records review, which runs in the first week and is delivered as a written report with a prioritised roadmap.

- Every system in use, with its name, what it is used for, who the vendor is and what is paid for it: clinical records, billing, accounting, pharmacy, payroll, point of sale, appointments, messaging
- Read access for the review, or an administrator walkthrough of each
- Where each system is hosted, who administers it, and who holds the administrator credentials
- **The backup position:** what is backed up, to where, how often, when a restore was last tested, and by whom
- Who has access to patient records, at what level, and whether access can be traced to an individual
- How credentials are shared today, including any account used by more than one person
- The network: internet provision, router and wifi administration, whether the patient wifi is separated from the clinical network, and what is on the network besides computers
- Devices: every computer, tablet and phone used for clinical or financial work, who owns it, and whether it is encrypted and patched
- Any record held only on a personal phone, personal email account or personal drive
- The website: who built it, who hosts it, who holds the domain and the registrar login, and who can publish to it
- The booking route: what a patient actually does to make an appointment today, and who sees it when they do
- Any prior incident: a lost device, a compromised account, a ransomware demand, or records that went missing
- Google Business Profile, Google Ads and Meta Business accounts: who owns them, who has admin, and what has been spent

M. Brand, marketing and what has already been tried

- Every brand asset held for both names: logo files, colour and type specifications, photography, and who owns the copyright



- The signage, print and stationery currently in use, and how much of it still carries the old name
- All social media accounts for both entities, with follower counts and admin access
- **HERcast:** episode list, publishing cadence, listener numbers by platform, where it is hosted, who edits it, and any sponsorship or commercial arrangement
- Everything spent on marketing in the last 12 months, by channel, and whatever is known about what it produced
- Any agency, freelancer, photographer or media contract, current or lapsed, and what is owed
- Any press coverage, award, speaking appearance or media list already held
- Referral sources as they are today: which GPs, physiotherapists, diagnostic centres or individuals send patients here, how many each sent in 12 months, and whether anyone tracks it
- **Any referrer who has stopped**, if that is known, and what is understood about why
- What the enquiry volume looks like: how many people ask about a service each month, through which channel, and how many convert

What the first two weeks look like from your side

So that your team knows what is coming and nobody is surprised by a request.

When	What happens	What we need from the team
Friday 2 October	We arrive. Walkthrough of the whole premises with you, then with the front desk, theatre, ward, laboratory, pharmacy and the fertility unit. The technology and records review opens	An hour of your time, and permission for us to talk to anyone
Week 1	Records and systems reviewed. Revenue reconstructed from the ledger and traced to the bank. The baseline agreed with you. Payer file assembled and the introduction letter drafted for your signature	The eight priority items, and the shared folder populated
Week 1	Short conversations with staff, individually and confidentially. Nothing anyone says is attributed to them	Twenty minutes per person, scheduled around clinical work
Week 2	Patient journey traced end to end on live cases. Pharmacy and stock counted. Findings tested with you before anything is written	Access to the day as it actually runs
End of Week 2	Written diagnostic delivered: what Arabella earns, where it leaks, what the operation can carry, and what to fix in what order. Technology and records report with a prioritised roadmap	A session with you to walk through it

Two working practices, so that they are agreed rather than assumed. We will ask staff questions directly and confidentially, because a team tells an outsider things it will not put in front of the founder, and none of it is reported back with a name attached. And we will trace real patients through the real process rather than sampling broadly, because ten episodes followed completely tells you more than a hundred counted partially, and every gap we find that way is specific rather than theoretical.

One last thing

If something on this list does not exist, the honest answer helps us more than a reconstruction. If something exists but is embarrassing, send it anyway. We have not yet audited a healthcare business in Nigeria where everything was in place, and the ones that improved fastest were the ones that showed us the real thing in week one.

Anything at all, call me directly.



Dr Debo Odulana

Founding Partner, Consult for Africa